



Endoscopy Report – EGD and Colonoscopy with Biopsies

Facility: Thornhill Endoscopy Centre (Affiliated with NorthStar Health Network)

Procedures: EGD + Colonoscopy (CSP)

Date of Procedure: 2022-01-20

Endoscopist: Gastroenterology (network affiliate)

Specimens: Gastric antrum/body biopsies; terminal ileum biopsies

Disposition: Home same day with standard post-procedure instructions

****Indication:**** Evaluation of gastrointestinal tract given ongoing systemic workup for chronic lymphadenopathy and concern for inflammatory or malignant pathology. Family history includes ulcerative colitis (per longitudinal notes).

****Procedures:**** Esophagogastroduodenoscopy (EGD) and colonoscopy with terminal ileum intubation. Conscious sedation administered. Continuous monitoring. No immediate complications.

****EGD Findings:**** Esophagus normal without erosions, rings, or varices. Stomach: mild antral erythema compatible with ****mild chronic gastritis****; biopsies obtained from antrum and body. No ulcers or mass lesions. Duodenum normal to second portion; biopsies not obtained from duodenum.

****Colonoscopy Findings:**** Cecum reached; withdrawal performed with careful mucosal inspection. Colonic mucosa grossly normal without polyps or masses. Terminal ileum intubated ~10 cm; mucosa demonstrates subtle erythema and granular change without deep ulceration. Biopsies obtained from terminal ileum.

****Pathology:**** (1) Gastric antrum: mild chronic gastritis; ****no Helicobacter pylori**** identified. (2) Terminal ileum: ****mild chronic active ileitis****. (3) No dysplasia or malignancy identified in submitted specimens.

****Assessment/Comment:**** Mild ileitis can be nonspecific and may relate to infection, NSAID exposure, or early inflammatory bowel disease; clinical correlation required. In the absence of diarrhea, abdominal pain, or weight loss, immediate escalation may not be required.

****Recommendations:**** Follow with primary/internal medicine for symptom correlation and biomarkers. If new GI symptoms arise, consider fecal calprotectin, repeat inflammatory markers, and gastroenterology follow-up.



Addendum / Reporting Notes

This document uses standardized institutional templates to emulate real-world medical record formatting for prototype testing. All names, identifiers, and institutional details are fictional. Clinical content is synthesized from the provided longitudinal summary.

If results appear internally inconsistent across documents, this is intentional to mimic typical real-world fragmentation. When discrepancies arise, the recommended approach is to reconcile using original source dates and the most recent verified medication/problem lists.